

Sepsis Recognition and Management

Early Identification, Bundles, and Outcomes
Monitoring

Regulation: 42 CFR §482.21 | SEP-1 CMS
Core Measure

POLICY OVERVIEW

Sepsis is a life-threatening emergency that requires rapid recognition and evidence-based intervention. CMS SEP-1 is a core quality measure tracking compliance with sepsis care bundles. Early identification and protocolized care significantly reduce sepsis mortality.

KEY REQUIREMENTS

1 Sepsis Screening

All clinical staff must be trained to recognize signs and symptoms of sepsis. Sepsis screening tools must be applied at triage, admission, and with any change in clinical condition. Electronic health record-based sepsis alerts support but do not replace clinical assessment.

2 3-Hour Bundle Compliance (Severe Sepsis)

Within 3 hours of severe sepsis recognition: obtain blood cultures (before antibiotics), measure lactate level, administer broad-spectrum antibiotics, and provide 30 mL/kg crystalloid bolus for hypotension or lactate ≥ 4 mmol/L.

3 6-Hour Bundle Compliance (Septic Shock)

Within 6 hours of septic shock recognition: apply vasopressors for MAP < 65 mmHg, re-measure lactate if initial lactate ≥ 2 mmol/L, and reassess volume status and tissue perfusion with documented reassessment.

4 Escalation and Notification

Defined escalation protocols must ensure timely physician notification for sepsis recognition. Rapid Response Team (RRT) activation criteria must include sepsis indicators. Critical care consultation should be triggered for septic shock.

5 Antibiotic Stewardship Integration

Antibiotic selection for sepsis must balance broad empiric coverage with stewardship principles. Blood cultures must be obtained before antibiotics. De-escalation to targeted therapy must occur within 24-72 hours based on culture results.

COMPLIANCE NOTES

SEP-1 compliance is publicly reported on Hospital Compare and affects Hospital Value-Based Purchasing (VBP) scores. A multidisciplinary sepsis committee should oversee the program, review cases of non-compliance, and monitor outcomes including mortality and length of stay.

COMMON SURVEY CITATIONS

Required under CMS SEP-1 Core Measure, and §482.21 – QAPI monitoring of sepsis outcomes.